Which treatment this guide covers This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

NCCP 00353 · VERSION 5

Osimertinib monotherapy

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

Make treatment fit into daily life

Take at the same time each day, with or without food. Swallow whole with water; do not crush, split or chew. If you forget a dose, take it when remembered unless less than 12 hours remain before the next dose, when it should be skipped. Confirm the instruction with your pharmacist if the next dose is exactly 12 hours away. Do not take an extra dose without advice if you vomit. Ask for the approved water-dispersion instructions if swallowing is difficult.

Before starting

Confirm the exact indication and mutation, dose, baseline tests, first review and emergency contact.

Taking the tablet

Take at the same time each day, with or without food. Swallow whole with water; do not crush, split or chew. If you forget a dose, take it when remembered unless less than 12 hours remain before the next dose, when it should be skipped. Confirm the instruction with your pharmacist if the next dose is exactly 12 hours away. Do not take an extra dose without advice if you vomit. Ask for the approved water-dispersion instructions if swallowing is difficult.

Supplies and visits

Arrange repeat prescriptions before tablets run out. Keep blood and ECG appointments. The oral regimen does not itself need an infusion line or pump.

If treatment is held

Ask why it is held, what must recover, when review is due and who will authorise restarting. Do not choose between 80 mg and 40 mg yourself.

At completion or change

Agree scans, symptom follow-up, the medicine plan and contraception after the final dose.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00353 v5 · official regimen
- EMA Tagrisso product information (updated 18/08/2026)
- NCCP NCIS antiemetics V8 (2025): parenteral scope; oral support individualised
- ADAURA final overall survival, Tsuboi 2023
- FLAURA final overall survival, Ramalingam 2020
- AURA3 primary progression-free survival, Mok 2017
- AURA3 final overall survival, Papadimitrakopoulou 2020
- NCCP SACT Antiemetic Guidance V6 (2025), Table 5 oral support and Table 8 risk

Source reconciled; independent clinical, pharmacy and patient/PPI review pending